Competitive Landscape: HER2-positive breast cancer (EU, 2023)

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Market Overview

In the EU HER2-positive breast cancer market, 1 brands compete for first-line+ patients as of Q4 2023. The segment is shaped by guideline updates, reimbursement decisions and real-world evidence.

The number of branded options in HER2-positive breast cancer has grown since 2021, increasing pressure on standard of care positioning.

Positioning: Enhertra

In the EU HER2-positive breast cancer market, Enhertra maintains a 36% market share as of Q2 2023, with the strongest uptake among oncologists.

Sentiment index for Enhertra in EU: 0.70 (scale 0 to 1, 2023). Clinicians view the brand strongly positively; infusion logistics add to treatment burden.

Competitive Dynamics

Enhertra has been gaining ground since its 2020 approval for HER2-positive breast cancer, pressuring Enhertra in first-line use.

Price competition, biosimilar entry and head-to-head evidence are expected to reshape formulary position over the next 24 months.

Dose modification guidance for Enhertra is generally followed, but oncologists said that practical monitoring of rash depends on local capacity and patient travel distance.

Subgroup analyses suggest that benefit with Enhertra is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Respondents expect real-world persistence on Enhertra to be lower than in trials, mainly because of discontinuation after early adverse events and the complexity of managing diarrhoea.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Treatment burden, including monitoring visits and discontinuation after early adverse events, was described as a key factor in long-term adherence and persistence.

Clinical guidelines in Italy recommend second-line use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Safety monitoring focused on rash and diarrhoea, with dose interruption or discontinuation used for severe adverse events.

Outlook

We expect continued consolidation in HER2-positive breast cancer. Differentiation will depend on tolerability, dosing convenience and evidence of overall survival or clinical benefit.

Peer-to-peer education and congress data were cited as the most trusted sources when oncologists evaluate new evidence on HER2 antibody-drug conjugate agents.

Safety monitoring focused on infusion reactions and diarrhoea, with dose interruption or discontinuation used for severe adverse events.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Oncologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between HER2 antibody-drug conjugate options.

Subgroup analyses suggest that benefit with Enhertra is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Sequencing after progression remains an open question; oncologists in Spain reported using a different mechanism of action after Enhertra fails.

Respondents expect real-world persistence on Enhertra to be lower than in trials, mainly because of treatment burden over long durations and the complexity of managing diarrhoea.

The unmet need in HER2-positive breast cancer persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

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